PM-04 Pediatric Pain Control

APPROVED CONTENT

Claiborne County EMS | Patients age 2 through younger than 16 years

Medical-director approval: October 8, 2026

Purpose

Use this pathway for acute pain in patients age 2 through younger than 16 years. Use actual weight when available, otherwise an approved length-based estimate. Treat immediate threats and the cause of pain without delaying transport.

Age boundary

UP-11 Adult Pain Control is for patients age 16 years or older. Do not use UP-11 for a patient younger than 16 years. Patients younger than 2 years require Medical Control for analgesic direction.

Quick flow

1 Assess

Confirm age and actual or length-based weight. Assess pain, airway, breathing, perfusion, mental status, allergies, and prior analgesics.

2 Treat threats first

Airway care, hemorrhage control, wound care, splinting, positioning, ice or heat when appropriate, calm reassurance, and rapid transport.

3 Oral route safe?

If alert and able to swallow and protect the airway: use ONE oral nonopioid with the approved weight-based dose and maximum.

4 Severe pain / oral route unsafe?

Paramedic: fentanyl 1 mcg/kg slow IV/IO ONCE; maximum 50 mcg. Medical Control is required before a second dose.

Standing-order medications

Do not routinely combine acetaminophen and ibuprofen

Medication	Dose	Key safeguard
Acetaminophen	15 mg/kg PO once; maximum 650 mg.	Use only if oral route is safe. Check liver disease, prior dose, and maximum daily exposure.
Ibuprofen	10 mg/kg PO once; maximum 400 mg.	Use only if oral route is safe. Avoid NSAID allergy, renal disease, dehydration, GI bleeding, and recent NSAID use.
Fentanyl	Paramedic: 1 mcg/kg slow IV/IO once; maximum 50 mcg.	Medical Control must authorize a second dose. Monitor airway, ventilation, oxygenation, mental status, and blood pressure.

PM-04 does not authorize ketorolac, morphine, hydromorphone, or ketamine as standing-order pediatric analgesics.

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Provider-level actions

EMT

Confirm weight source; assess and reassess pain using a developmentally appropriate measure; provide nonpharmacologic care; and give one approved oral nonopioid when the oral route is safe and there is no medication-specific contraindication.

AEMT

Perform all EMT care. Establish IV/IO only when clinically indicated. No additional AEMT standing-order analgesic is authorized by PM-04. Request Paramedic support or contact Medical Control when needed.

Paramedic

Perform all prior care. For severe pain or an unsafe/inadequate oral route, give fentanyl 1 mcg/kg slow IV/IO once, maximum 50 mcg. Contact Medical Control before any second fentanyl dose.

Safety checks before and after medication

- Do not delay airway management, hemorrhage control, shock treatment, splinting, or time-sensitive transport to complete analgesia.
- Do not give oral medication when airway protection or swallowing is impaired, active vomiting creates an aspiration risk, or the prior dose/time is uncertain.
- After fentanyl, monitor airway, ventilation, oxygenation, mental status, and blood pressure. Keep suction and ventilation equipment immediately available.
- Reassess pain, respiratory rate and effort, perfusion, mental status, vital signs, and treatment response after every intervention and at handoff.
- Document the weight and source, medication, dose, route, time, pain assessment, response, adverse effects, and Medical Control direction when used.

Operational notes

Use patient self-report when reliable or a developmentally appropriate observational pain scale. The goal is meaningful improvement in comfort and function, not complete elimination of pain before transport.

Medical Control trigger

Contact Medical Control before any second fentanyl dose and for patients younger than 2 years, persistent severe pain despite the pathway, atypical medication risk, or any situation outside this protocol.

Related pathways

- UP-11 Adult Pain Control - for patients age 16 years or older only.
- Medication Reference - verify current formulary information and medication-specific cautions.

Source: Claiborne County EMS medical-director approval, October 8, 2026. App content remains subject to agency release governance.